

OPIOID USE DISORDER AMONG OLDER ADULTS: ADDRESSING MULTIMORBIDITY AND GERIATRIC CONDITIONS

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Dr. Han will present multidisciplinary research focused on understanding the healthcare needs of older adults with opioid use disorder. Nationally, there is a sharp increase in older adults with opioid use disorder (OUD) and the number of older adults dying of opioid-related overdoses. This lecture will first describe the prevalence of geriatric conditions and comorbidities among older adults who receive care in opioid treatment programs. Then, Dr. Han will present patterns of acute healthcare utilization among older adults with opioid use disorder. Qualitative data from patient experiences of aging with opioid use disorder and the challenges of managing chronic diseases will then be presented. The lecture will conclude with a discussion about opportunities for developing multidisciplinary models of care to best deliver geriatric-based interventions for this population.

SESSION 3000 (PAPER)

ASSESSMENT AND MEASUREMENT

CONSUMERS AND PROVIDERS ON THE NEEDS OF LONG-TERM SURVIVORS AND PEOPLE AGING WITH HIV IN NEW YORK STATE

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Objectives: To document the practical needs, and develop quality initiatives to address those needs, of the growing population of long-term survivors (LTS) and older people with HIV (OPH) in New York State.

Methods: The HIV+ Aging/LTS/Perinatally Diagnosed Subcommittee of the NYS Quality of Care Consumer Advisory Council used community based participatory research methods to design a statewide survey based on categories identified in August 2020 virtual town halls with consumers and providers across New York. Syracuse University launched the survey, open to consumers aged 18 and over who were LTS or OPH, clinicians, and social services providers, in June 2021 using Qualtrics™. Participants chose the three most important barriers and recommendations for each category. Responses were characterized using basic descriptive statistics.

Results: Participants included 124 consumers from 26 counties, 20 clinicians, and 24 social service providers. Two thirds of participants were cisgender men (67%), 27% were

African American, 80% identified as both LTS and OPH. On average, consumers were 58 years old, had been living with their HIV+ diagnosis for 27 years, and reported 4 additional conditions, most commonly depression (30%). LTS and OPH were concerned about clinical and financial needs, particularly coordination of clinical care, unmet housing needs, cultural representation in mental health services, and financial support of LTS and OPH. Implications: Community based participatory research can inform and stimulate changes in clinical care for LTS and OPH. Survey results are informing a plan for functional screening of OPH and LTS that can be performed by certified peer workers.

MODE EFFECTS ON COGNITIVE FUNCTIONING ASSESSMENTS IN THE HEALTH AND RETIREMENT STUDY

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As the population of the US ages, there is interest in assessing health conditions associated with age and longevity, such as age-related decline in cognitive functioning. As a result, there is an increased focus on measuring cognitive functioning in surveys of older populations. One challenge relates to conducting comparable measurement across survey modes (e.g., phone vs. web). Compounding this is that mode of survey administration is often not assigned randomly making inter-group comparison more difficult. This paper addresses these issues using a novel experiment embedded within the Health and Retirement Study (HRS). The HRS, a US-based cohort of people over 50, has measured cognition since its inception using both in-person and telephone modes. In 2018, a sample of approximately 3700 respondents was identified as web-eligible based on a prior report of internet access along with other selection criteria. Of these, 60% were randomly selected for the web sample with the remainder serving as controls, assigned to telephone mode for comparison purposes. We deploy techniques from item response theory (IRT) and differential item functioning (DIF) to estimate the difference in cognitive functioning between web and phone respondents in 2018 based on longitudinal cognition data collected prior to 2018. Second, we estimate the overall effect of taking the survey via the web as compared to the phone. Third, we examine item-level variation in the magnitude of the mode effect and suggest possible methods for adjustment to support longitudinal consistency. These results are important in guiding future research that utilizes web-based cognitive measures.

PERCEIVED AND OBJECTIVE EXECUTIVE DYSFUNCTION JOINTLY PREDICT BRAKING REACTION TIME IN OLDER DRIVERS

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